

Phase 1 Strategy

To: Lena Whitford, CPO, Fulcrum Health

From: Xio Santaella, Sr. UX Designer, PressW

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Fulcrum should start with the clinician mobile experience. The scheduler concern is real, but the larger risk is competitive erosion. Phase 1 should improve the clinician experience without disrupting the scheduler workflows currently protecting renewals.

Discovery Approach: First 30 Days

Three questions drive discovery: where is the clinician experience actually failing, which constraints are technical versus UX-related, and which workflows cannot safely change.

There is no dedicated researcher on this engagement. Discovery falls to the UX lead. Three workstreams run in parallel:

- **Inside PressW:** Audit RFP feedback and support signals. Separate evidence from assumptions. Define Phase 1 success before opening Figma.
- **Inside Fulcrum:** Engineering sessions with Tomas on rules-engine constraints. Alignment with Jules on product assumptions. Protocol with Aisha on realistic research access.
- **Customer side:** 5 to 7 clinician interviews on mobile workflows. 2 to 3 scheduler sessions with power users to map what cannot break.

By day 30: a clinician friction map, a technical constraints brief, an account-risk assessment, and a prioritized Phase 1 recommendation.

Where the Leverage Is

Mobile, clinicians, first. Every signal points the same direction:

- **WAU:** 38% against a 70% benchmark
- **NPS:** 42 in 2022 to 19 today, driven by clinician feedback
- **RFP verbatims:** 'clunky,' 'looks dated' - two consecutive losses, same words
- **Swap funnel:** 64% abandon the in-app swap and text the scheduler instead. Only 22% complete it in the product
- **Field:** one scheduler takes 30 texts a day from clinicians who cannot complete tasks in the app. The scheduler has become the workaround

The app handles lookups fine. It fails on every action: swapping, time off, availability. A redesign scoped only to the lookup case will not move NPS. If clinicians stop routing through schedulers, schedulers also get meaningful hours back - a business case upside that has not been quantified.

One scheduler put it directly: clinicians are the ones who get pulled into RFP evaluations. Schedulers use the system every day but clinicians form the opinion that ends up in the boardroom. That is exactly where Fulcrum is losing.

There is a technical reason to start here too. The mobile app runs on a separate API layer that does not require touching the rules engine. It is the lowest-risk path to a visible improvement.

What I Would Redesign, Keep, and Cut

- **Redesign:** Clinician mobile navigation, information hierarchy, and action flows - swap, time off, availability. Not a cosmetic refresh. Reducing friction in repeated daily tasks.
- **Keep:** The scheduler console. 80 hospital systems with individual configurations is not a place to move fast. Stability is a feature.
- **Cut:** Broad platform work, low-frequency admin tooling, net-new features. The instinct when products feel dated is to add. Usually the problem is complexity, not missing features.

Risks and Unknowns

- **The problem may not be visual.** The calendar color feature looks like a time-off request but is not. One clinician missed a family event because of this. That is a model problem - a visual redesign will not fix it.
- **The swap workflow is dangerous to touch.** There are five config-dependent swap workflows and schedulers have built invisible workarounds for all of them. The Approve button does not move the shift - four additional steps required, nothing tells you. This is partly a rules engine issue. Engineering must be in the room for every swap decision.
- **Hard constraints:** Login is already being rebuilt - do not design around it. Schedule grid is off limits. ACGME rules apply at ~20% of accounts. Engineering needs 4 to 8 weeks lead time for anything touching the rules engine.
- **Research access:** Clinician access takes 2 to 3 weeks and requires account coordination. Without a protocol with Aisha's team from day one, discovery time goes to logistics instead of learning.

Before locking scope: why is clinician mobile engagement actually low, and which constraints are engineering-side versus UX-side. Those answers either validate this strategy or change it.

See you Wednesday.

User Stories

Product: Fulcrum Health Mobile App

Focus: Clinician mobile experience - Phase 1

Format: Job stories - When / I want to / So I can

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These stories cover the clinician mobile experience identified as the Phase 1 focus, and the scheduler workflows that Phase 1 must not break.

1. Clinician checking their schedule between patients

Clinician / Mobile user

When I have a few minutes between appointments and need to know what is coming next,

I want to see my remaining schedule clearly without digging through the app,

So I can stay oriented without losing time I should be spending with patients.

We know it worked when: Clinicians complete this task on mobile in under 60 seconds without switching to desktop.

2. Clinician requesting time off on mobile

Clinician / Mobile user

When I need to request time off and I am away from a desktop,

I want to find the actual time-off request flow and submit it without confusion,

So I can have my request recorded correctly without relying on a workaround or texting the scheduler.

We know it worked when: Time-off requests submitted via mobile increase. Support tickets for missed or misrouted requests go down. The calendar color feature no longer causes false submissions.

3. Scheduler handling a same-day cancellation

Scheduler / Desktop user

When A clinician cancels last minute and there is an open slot in the schedule,

I want to find a replacement and confirm the change without disrupting the rest of the day,

So I can keep the schedule covered without escalating to anyone.

We know it worked when: Schedulers resolve same-day changes without support tickets. This workflow is untouched by Phase 1 mobile changes.

4. Clinician shows up to a shift they thought they swapped off

Clinician / Mobile user - unhappy path

When I submitted a swap request and the app told me it was approved,

I want to trust that the swap is actually reflected in my schedule,

So I can not show up to a shift I thought I was off, or miss one I thought I was covering.

We know it worked when: Zero instances of a swap marked approved that does not update the clinician schedule. Clinicians do not need to call the scheduler to confirm a swap actually went through.

5. New clinician trying to use the app for the first time

Clinician / Mobile user - unhappy path

When I am new and opening the app for the first time without any guidance,

I want to find my schedule and understand what I can do without a tutorial or texting a colleague for my org code,

So I can get oriented and into my workflow without the tool being the first obstacle of my new job.

We know it worked when: New clinicians complete their first core task without contacting IT, the scheduler, or a colleague. Orientation no longer requires a dedicated app tutorial.
